



Management of webPAS Patient Alerts for Workplace Violence and Aggression and Associated Risks (Acute) Policy

Preamble

Consumer Participation

The Rockingham Peel Group (RkPG) recognises and fully supports the need for the patient and/or their primary carer to¹:

- Know and exercise their healthcare rights.
- Be engaged in their healthcare.
- Have access to information about treatment options.
- Participate in treatment decisions.
- Have access to information about agreed treatment plans.

Scope of Practice

It is the responsibility of each health care provider to ensure all patient care and therapeutic interventions they provide are within their individual scope of practice. Refer to the [RKPG Policy Writing Guide Summary](#) for further information about Scope of Practice.

Purpose

The purpose of this policy is to outline the indication for, and management of patient alert flags related to aggression, violence and children at risk in the webPAS system.

1. Background

1.1 About this document

- The RKPG is committed to preventing and reducing the risk of workplace violence and aggression. The RKPG supports the promotion of a comprehensive strategic approach to the prevention and management of workplace violence and aggression which is outlined in the [RKPG Prevention and Management of Violence and Aggression \(Acute\) Policy](#) and the [South Metropolitan Health Service \(SMHS\) Workplace Aggression and Violence \(WAV\) Strategy](#).
- The RKPG acknowledges that all workers, internal and external consumers of the service have a right to a safe and secure environment. It recognises the risk to workers from violence and aggression and its potentially damaging effects on the individual, work performance and organisation. The RKPG will not tolerate any form of violence, against workers, or its consumers, and believes that such behaviour is unacceptable, irrespective of the form it takes or whatever reasons are given for it.
- The RKPG believes that all workers, service users and visitors, have a duty to treat each other with dignity and respect, and to behave in an acceptable and appropriate manner. Workers have a right to work, as service users have a right to be treated, free from fear of assault and aggression in an environment that is safe and secure.

RESPONSIBILITY				NATIONAL STANDARDS
First issued	May 2024	Revisions	NA	National Standards 1, 5, 6
This version	Apr 2026	Revision Due	Apr 2029	
Responsibility of	Co-Directors Medical, Surgical & Mental Health	Endorsed by	DONM/DCS	
Title	Management of webPAS Patient Alerts for Workplace Violence and Aggression (Acute) Policy			

- The causes of workplace violence and aggression are varied and complex. They are usually the result of the inter-relationships of a number of factors relating to the individual, the physical and organisational environment and behavioural responses of staff.
- RkPG and SMHS are required to adhere to the Bilateral Schedule and [CAHS Guidelines to Protecting Children \(2020\)](#). HSPs and community have a joint role in achieving safe outcomes for infants, children and parents. When a child has been deemed at risk of abuse or neglect, Communities may request a Whole of Health alert outlining the actions required to protect the safety and wellbeing of children. This is in line with the Bilateral Schedule: Interagency collaboration processes in the pre-birth period or on the birth of a child, when they are identified as at risk of abuse or neglect and CAHS Guidelines to Protecting Children (2020).

1.2 Role of webPAS

- webPAS is the current Patient Administration System (PAS) that is used state-wide for all Western Australia (WA) Health Sites. The webPAS system delivers patient data to a variety of clinical and non-clinical applications used throughout WA Health. Risk alerts need to be added to webPAS to ensure clear communication and recognition of risk throughout all clinical areas.
- The intention of this policy is to record relevant behaviour-based risk alerts in webPAS to reflect the risk presented by a patient using the health service. These alerts will assist in the management of patient care planning and delivery.

1.3 Key definitions

Term	Definition
Workplace violence and aggression (WVA)	Any incident where a worker is abused, threatened or assaulted in circumstances arising out of, or in the course of, their employment. Examples include, but are not limited to verbal, physical or psychological abuse, threats, spitting, biting or throwing objects. It can be perpetrated by patients, relatives, members of the public or other workers.
Whole of Health (Child at Risk) Alert	A Whole of Health Child at Risk Alert is the statewide patient alert (for a child at risk) for public hospitals. The risk alert outlines the actions required to protect the safety and wellbeing of children at risk. For the purpose of this guideline the term 'child' is anyone under the age of 18 years of age and includes unborn children.
PCBU	A person conducting a business or undertaking, in this instance, the Rockingham Peel Group.
Worker	A person is a worker if the person carries out work, paid or unpaid in any capacity for the PCBU. Workers include employees, trainees, apprentices, work experience students, volunteers, outworkers, self-employed person, contractors or sub-contractors, employees of a contractor or sub-contractor, employees of a labour hire company and a person of a prescribed class.
Workplace	An area or place or vehicle, where a worker goes or is likely to be while work is carried out on behalf of the RKPG and includes but is

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	not limited to hospital premises, laboratories, workshops, training rooms, on-site and off-site facilities and offices.
webPAS	web-based Patient Administration System.
Patient alert or webPAS alert	A specific entry and or warning in the webPAS application that outlines the type and nature of the alert recorded.
Senior clinician	This description includes various staff members who are authorised to approve the addition of webPAS alerts to patient's records. These roles include but are not limited to: The Treating Medical Team, the Senior Nurse in the Area (Applies to SRN 3+), Senior Social Worker
ARARG	Aggression Risk Alert Review Group.
Whole of Health Alerts Group (RkPG)	Review group of Whole of Health alerts which consist of Social Work Coordinator, Senior Social Worker Maternity and Senior Social Worker Emergency department.

2. Governance responsibilities

RKPG and sites have various committees to manage workplace violence and aggression. These committees work in consultation and collaboration with internal and external stakeholders to reduce instances of workplace violence and aggression and associated risks.

2.1 Roles and responsibilities

- **Officers:** The responsibility for the implementation and management of this policy lies with the Executive. The Executive will be responsible for ensuring that appropriate arrangements and funding are in place for the prevention of violence and promotion of safe working in all its operations and for systematic implementing and monitoring procedures.

2.2 Staff access to health records including webPAS

- Health records may only be accessed by staff who:
 - * Are involved in the clinical care of the patient.
 - * Have obtained necessary approval to access health records for research and/or education purposes.
 - * Require access to the patient record in the course of their work duties i.e. health records management staff.
- All staff are to adhere to Department of Health (DoH) requirements relating to patient confidentiality and ensure they take all necessary precautions to ensure patient privacy and confidentiality of health records is maintained.
- Refer to the [SMHS Health Record Access, Use and Disclosure Policy](#).

3. Use of webPAS

3.1 Use of webPAS alerts to communicate potential risk

- The use of webPAS alerts ensures that other users are aware of a patient's history or current management plan. In particular, patients with recorded events of aggression

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or violence can have these documented in the webPAS system. Patients defined as “Children at Risk” can also have this documented in the webPAS system.

- Only approved patient flags can be added to the webPAS system.
- Importantly, behaviour management plans and practices must be in place to appropriately manage the patient during their admission. Where applicable, it is important that staff are engaged with the patient in managing WVA risks. This should include informing the patient that continued WVA related behaviour may result in a webPAS alert being added to their records. When a patient is transferring or changing locations, the handover must include any risks related to WVA and what controls or plans are in place.
- HIMS clerks are to print off webPAS WVA alerts on to lavender paper and highlight the patient’s identification labels in purple to ensure the alert is visible throughout the patients visit.

Note: The webPAS alert does not replace the requirement for behaviour management and is only a support mechanism to communicating, preventing and managing violence and aggression related risks.

3.2 Indications for recording a patient alert in webPAS

- When considering a webPAS alert, the following risk criteria shall be considered. These should be seen as minimum criteria and multiple criteria may be applicable:
 - * Physical aggression with intent causing actual bodily harm requiring medical attention.
 - * Unintentional aggression causing actual bodily harm requiring medical attention.
 - * Assault on staff that may result criminal charges.
 - * Threatening with a weapon, actual assault with a weapon or weapons removed.
 - * Code Black (for Personal Threat) resulting in physical restraint.
 - * Intentional aggression causing biological risk includes biting and spitting.
 - * Verbal threats directed at an individual/s intending significant harm or death.
 - * Sexual assault, harassment or stalking behaviour.
 - * Significant history of risk criteria within the last 2 years.
 - * Behaviour is ongoing, escalating and likely to be repeated.
 - * Child at Risk notification from the Department of Communities Child Protection and Family Support (CPFS) has occurred.
- It is important to recognise that during a patient’s admission, the whole patient journey shall be considered as part of the risk assessment.

Workplace Violence and Aggression Alert Process

4.1 Process to initially add an alert related to Workplace violence and aggression

- To ensure that any potential risks are identified and communicated promptly, the work area senior clinicians are to complete a RKPG webPAS Patient Flag Notification eForm.

RESPONSIBILITY				NATIONAL STANDARDS
First Issued	May 2024	Revisions	NA	
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Responsibility of	Co-Directors Medical, Surgical & Mental Health	Endorsed by	DONM/DCS	
Title	Management of webPAS Patient Alerts for Workplace Violence and Aggression (Acute) Policy			

- The overall process is outlined in Appendix A: webPAS Patient Flagging Process for Violence and Aggression alerts and the specific process for adding an alert in webPAS is documented in section 4.2. Use of webPAS to capture alerts.
- The decision to add a webPAS alert should be considered against the criteria listed in section 3.2. above.
- Once the form is completed, reviewed, and approved. The [RKPG WVA webPas Patient Flag Notification](#) form is sent to RKPG Systems Support to add the endorsed patient aggression alert to webPAS.
- This form can be located here: [RKPG WVA WebPas Patient Flag Notification](#)

Figure 1 - QR Code for Notification Form



4.2 Processing and evaluation of an alert request for Workplace Violence and Aggression

- To provide governance to this process, all requests shall be reviewed and considered by the Aggression Risk Alert Review Group (ARARG). A preliminary review of the alert will be performed by the Coordinator and HIMS informed to place the alert if approved (request will be documented for ARARG review even if not approved in the preliminary stages). The ARARG will review the webPAS alert notification in combination with the patient's clinical records and will decide as to whether a formal webPAS alert is to be retained or removed from the patient's records.
- The Coordinator, will aggregate the RKPG webPAS Patient Flag Notification eForms and can request that a webPAS data export be provided listing any new alerts added for a specific period.
- Each location will have ARARG, however the composition of each group will vary depending on location, staff mix and scale of the operation.
- The ARARG will be coordinated at Rockingham General Hospital (RGH) and will review RKPG webPAS Patient Flag Notifications submitted at that same operation.
- The ARARG shall be coordinated by a single party.

Location	Coordinator role
RKPG	Nurse Educator – Management of Aggression or similar Senior Clinician

- In reviewing alert requests, the location coordinator, will undertake a review with relevant representatives. These representatives will vary depending on the patient and their presentation, history and care requirements.

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Representative	Situation or relationships
Consultant or Senior Medical Officer	From a team involved in the care of the patient or the same speciality/sub speciality.
Nursing Director or Coordinator of Nursing	From a team involved in the care of the patient or the same speciality or support/corporate service.
Psychiatrist or Mental Health Practitioner	From a team involved in the care of the patient or the same speciality/sub speciality.
Senior Social Worker or Social Work Coordinator	From a team involved in the care of the patient or the same speciality/sub speciality.
HIMS Representative	Site based.
Security Team Representative	Site based.
Aggression Prevention Specific Role	Site based.

- The Coordinator of the ARARG can seek additional expertise to assist with the assessment and review of aggression alerts out of session as indicated. This may include, but is not limited to, social work, community health, drug and alcohol services.
- Evaluation of webPAS alert notification, should only be completed after the patient has been discharged or during a protracted admission or length of stay period.
- Following evaluation of the alert notification, the ARARG will determine the duration that the alert will be applied for. The default validity period for a first alert is 12 months unless otherwise specified.

4.3 Considerations regarding decision to add an alert

- When reviewing the webPAS alert notification, the ARARG must assess the patient against the established criteria. Patients with acute and reversible conditions that result in aggressive behaviours should be reviewed to estimate the likelihood that repeated or ongoing aggressive behaviours will represent a risk to staff and other patients.
- Decisions regarding alerts should be documented on the RKPG webPAS Patient Flag Notification e-Form workflow.
- By way of an example, a simple acute delirium with aggression secondary to an infection may not warrant an alert being added, as once the infection is addressed, the aggressive behaviours are also resolved. However, in this same scenario, if there was ongoing or repeated aggressive behaviour after the resolution of the infection, then an alert may be indicated.

4.4 Retaining or removing existing alerts for Workplace Violence and Aggression

- ARARG will be responsible for the review of existing alerts in the system. This will require each location to establish and manage a process to ensure that alerts are reviewed. The guiding principle will be to determine what risk is represented by the patient for further acts of violence or aggression toward staff and or other patients or consumers.

RESPONSIBILITY				NATIONAL STANDARDS
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- In addition to this principle, these reviews should consider the patient's medical conditions, their level of control and other presentations during the period as part of any determination.
- During these reviews, the ARARG can make the following determinations:
 - * Retain the current alert for a fixed period.
 - * Retain the current alert indefinitely.
 - * Remove the alert.
- The reviews conducted by the ARARG must be documented and retained by each location.
- All records related to the webPAS alert request and review process shall be maintained with the patient record.

5. Child at Risk Alert Process

5.1 Process to add an alert related to "Child at Risk" (CAR)

- On identifying a child at risk (including an unborn child at risk), or observing an existing CAR alert, after establishing immediate safety, clinicians are required to actively assess whether referrals to additional services are required.
- Consider whether there is a need to share information with other RkPG staff e.g. Security
- Consider and address barriers to attending appointments etc.
- Social Work to confirm pregnancy.
- CAR Alert to be placed for 3 months.
- If the alert requires extension CPFS are required to send a new alert.
- Other actions will depend on the context of the health service setting, the management plan in place and the circumstances of the child or pregnant person. When a CAR Alert is activated, it is recommended that the Department of Communities, Child Protection (Communities) is made aware of the concerns.
- The CPFS whole of health alert is to be printed off and a patient sticker is to be placed on the alert. The whole of health alert and patient risk log is then to be given to the HIMS clerk for urgent scanning to DMR.
- HIMS clerks are to print off webPAS WVA alerts on to lavender paper and highlight the patient's identification labels in purple to ensure the alert is visible throughout the patients visit.

5.2 Processing and evaluation of a Child at Risk alert request

- Evaluation of the Child at Risk alert process takes place annually within the RkPG Social Work Department Protecting Children Guidelines and Processes.

5.3 Retaining or removing existing Child at Risk alerts

Children and adolescents who have been subject to abuse and neglect are likely to experience long term health and wellbeing concerns. It is recommended that CAR Alerts remain active on the child's record until 18 years of age if required as per Social Works professional judgement.

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RkPG staff are required to provide additional support for children and young people at risk as follows:

- All clinicians providing care for a child with a CAR Alert are to consider if the CAR Alert needs updating based on presentation and clinical judgement and escalate accordingly.
- A CAR Alert review may involve review of health service records, past and current presentations, and any other available information. This may include information from other providers and agencies.
- If 'Other' risk concern is noted in the Alert clinicians are to review the uploaded CAR Alert Notification Form for the details.
- If a review indicates an extension or update to the CAR Alert is required, complete a CAR Alert Notification Form.
- Once the child has birthed and CPFS have provided safety direction the alert can be inactivated by the Social Worker working with the family and child/ren. The Patient risk log is to be updated with a date inactive and scanned into DMR.

6. Process overview

6.1 webPAS patient record flagging process

Refer to the attached flow chart (Appendix A).

6.2 Use of webPAS to capture alerts

- Once an alert has been approved for addition to webPAS, this instruction will be provided to the webPAS administration team or equivalent at each location.
- To add an alert to webPAS, the following information will be required:
 - * **Alert type** - To ensure consistency of application, only four types of alerts will be added to webPAS. These are:
 - Patient – Physical.
 - Patient – Verbal.
 - Patient (P) – Possession of weapon.
 - Patient – Child at Risk
 - * **Date activated** – This will be the date of recording in webPAS.
 - * **Next review date**- This will be a default 12 months from the date activated.
 - * **Authorising clinician** – webPAS has a list of recorded providers. This must be confirmed by the ARARG.
 - * **Severity level** – This field will not be used.
 - * **Comment** – This is a character limited text box that will be used to include a brief history and include how the patient was managed. It is important to include location and relevant incident details.

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This version	Apr 2026	Revision Due	Apr 2029	
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Title	Management of webPAS Patient Alerts for Workplace Violence and Aggression (Acute) Policy			

Figure 2 - webPAS Patient Risk Alert Options

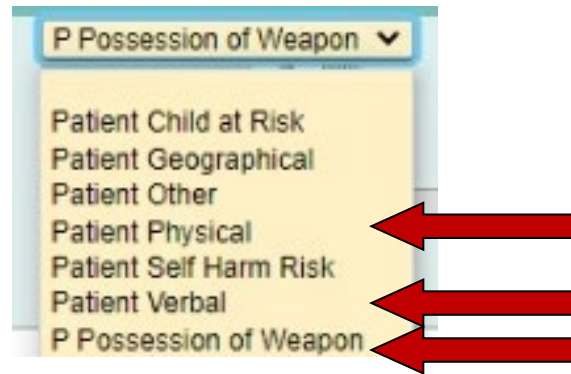


Figure 3 - webPAS Alert Recording Screen

Figure 4 – Example of webPAS Alert on a Patient File



- Following the recording of the alert, webPAS will display a black alert symbol in the header field of the webPAS record.

6.3 Existing alerts in alternative patient management systems

- There are two additional clinical information systems currently in use across SMHS. These are the Psychiatric Services On-Line System (PSOLIS) and the Emergency Department Information System (EDIS).
- The PSOLIS is a mental health clinical information database that has been developed for use in all government mental health facilities throughout WA. EDIS is a Clinical Information System used to record clinical treatment, monitor patient status, and track other associated activities of patients presenting to the Emergency Department.
- Both systems include functionality that allows 'alerts' to be added to patient records. In the event that patient has significant and contemporary alerts related to WVA,

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This version	Apr 2026	Revision Due	Apr 2029	
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these can be transferred to the webPAS system and the review timeline initiated as per the process.

6.4 Informing stakeholders in the process

- Once the decision to add an alert to the patient's webPAS record has been confirmed by the ARARG, this task will be undertaken by an appropriate webPAS Patient System Support team or equivalent at each location.

7. Data reporting

Periodic review of webPAS alerts (patient flag counts)

- Each location shall be required to maintain records for the alert requests that have been lodged. Additionally, each location must record the determinations made on these requests, plus records of review undertaken and their subsequent outcomes.
- This data shall be reported to the RKPG WAV committee on a quarterly basis.

Compliance monitoring

Each RKPG site or service Executive Director is to ensure compliance with this policy. Compliance monitoring is also to be managed via appropriate completion and review of Combined Hazard and/or Incident Report System (CHoIR) and/or Datix Clinical Incident Management System (CIMS). Additional recording of security related events will be maintained in Sentry.

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Title	Management of webPAS Patient Alerts for Workplace Violence and Aggression (Acute) Policy			

References

1. Australian Commission on Safety and Quality in Health Care (ACSQH). The National Safety and Quality Health Service Standards Safety and Quality Improvement Guide: Standard 1 Governance for Safety and Quality in Health Service Organisations. 2nd ed. (Version 2) 2021. Available from <https://www.safetyandquality.gov.au/our-work/assessment-to-the-nsqhs-standards/>

Acknowledgement

The RKPG WebPas Alerts for Workplace Violence and Aggression (Acute) Policy was created in collaboration with East Metropolitan Health Service, with acknowledgement and thanks.

Links

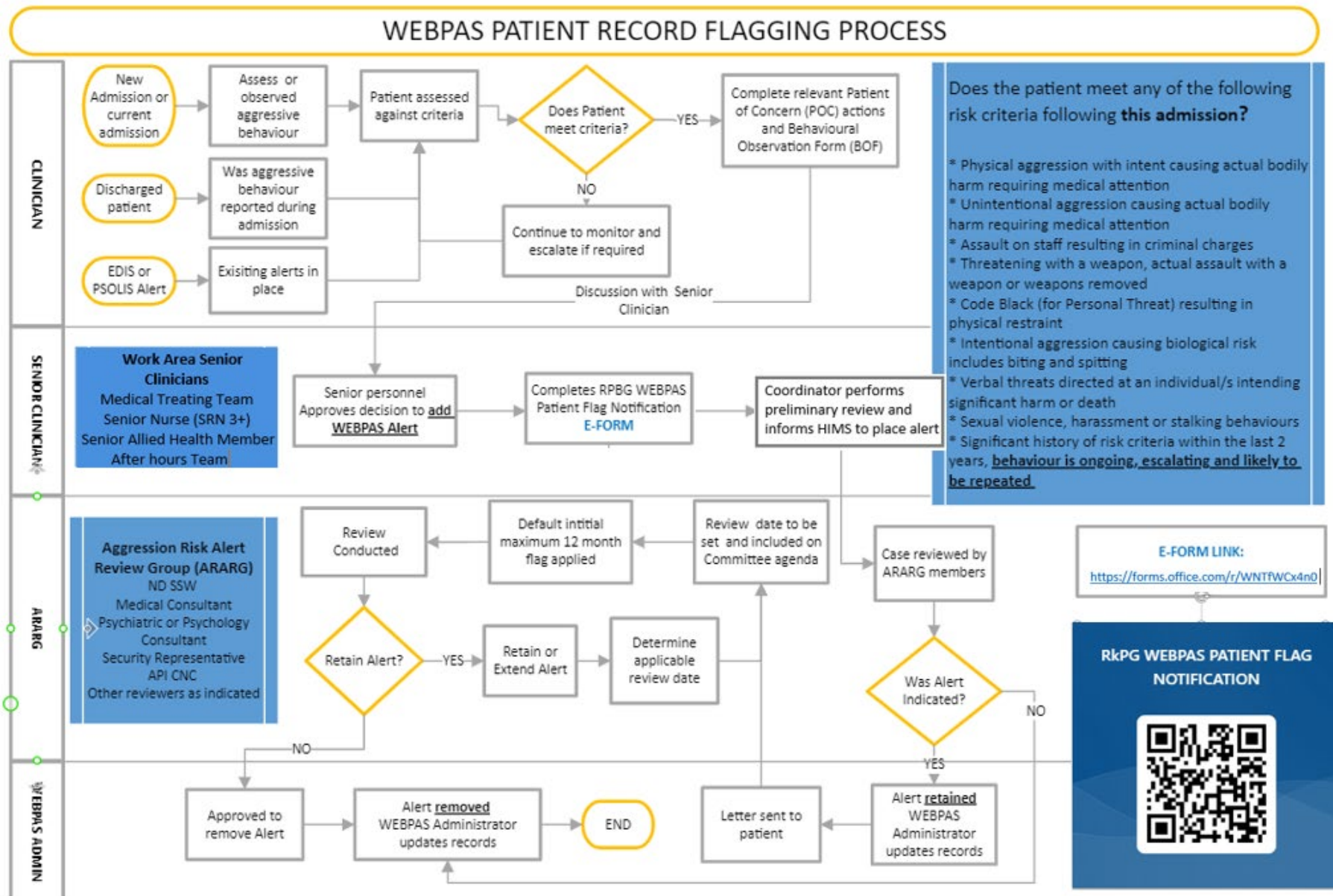
[DoH Clinical Incident Management Policy 2025](#)
[DoH Discipline Policy](#)
[DoH Notifiable and Reportable Conduct Policy](#)
[DoH Prevention of Workplace Bullying](#)
[RKPG Emergency Department Information System \(EDIS\) Alerts \(Corporate\) Policy and Procedure](#)
[RKPG Prevention and Management of Violence and Aggression \(Acute\) Policy](#)
[SMHS Health Record Access, Use and Disclosure Policy](#)
[WA Health Services Act](#)
[WA Mandatory Reporting of Notifiable Incidents to the Chief Psychiatrist. Psychiatrist.](#)
[WA Work Health and Safety Act 2020](#)
[WA Work Health and Safety Regulations 2022](#)

National Standards



RESPONSIBILITY				NATIONAL STANDARDS
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Appendix A: WebPas Patient Record Flagging Process for Violence and Aggression alert



RESPONSIBILITY				NATIONAL STANDARDS
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